

May 21, 2026

Hiring Team
AKASA

Re: AI Healthcare Operations — Remote

AKASA's premise—that AI for healthcare operations must be built by people who understand the operations, not just the algorithms—is the same conviction that led me to build my own AI platform. I am both a career RCM operator and a working AI builder, and that dual identity is rare and valuable. For 16 years, I have served as **Director of Revenue Cycle Operations at Dharma Solutions**, managing **120+ FTEs** and **\$500 million+ in annual revenue** with a **98% client retention rate**. I **reduced Days in A/R from 65 to 42** and **recovered over \$47 million in denied revenue**. These are not consulting recommendations on a slide—they are outcomes I produced by running real operations, processing real claims, and solving real problems that AI alone cannot solve without human domain intelligence behind it.

Understanding where AI fails in RCM is just as important as knowing where it succeeds, and I have learned both through direct experience. AI fails in RCM when it encounters edge cases that exist only in specialty-specific payer contracts, when it misinterprets ambiguous denial reason codes, when it generates appeals that are syntactically correct but clinically naive, and when it operates on incomplete or inconsistent data from legacy EHR systems. I **founded and architected Denial Doctor**, a HIPAA-compliant, multi-agent AI platform that deploys six specialized agents for denial classification, appeal drafting, payer follow-up orchestration, predictive scoring, root-cause analytics, and compliance monitoring. It delivers a **320% ROI and \$12 million in annualized savings**—not because the AI is magical, but because I designed every agent with the operational knowledge of where the process breaks, what data is reliable, and where human escalation is non-negotiable. This is exactly the kind of production-hardened AI thinking that AKASA needs.

I have hands-on experience with the integration layer that makes or breaks AI deployment in healthcare. My work with Denial Doctor involves FHIR R4 integration for clinical data ingestion, EDI 837/835 transaction parsing for claim and remittance processing, and HL7 interface handling for legacy system compatibility. I understand that the hardest part of deploying AI in healthcare operations is not the model—it's the data pipeline, the interoperability layer, and the compliance guardrails that ensure the AI's output can actually be acted upon within existing workflows. AKASA's customers don't buy AI for its own sake; they buy it because it makes their revenue cycle faster, more accurate, and less expensive. I speak that language natively because I have been the buyer, the builder, and the operator.

The healthcare AI space is full of companies building impressive demos that crumble in production. What differentiates AKASA is its commitment to AI that works in the real operational environment, and that commitment requires people who have bled in that environment. I have spent 16 years in the operational trenches and three years building AI that survives them. I bring the operator's skepticism about what AI can realistically achieve, the builder's ingenuity about how to push those boundaries, and the executive's accountability for making sure the technology actually delivers measurable ROI. For AKASA, that combination is not a nice-to-have—it is the foundation of product credibility.

I would welcome the chance to discuss how my dual expertise as an RCM operator and AI builder can contribute to AKASA's mission. For the full story behind my journey from operations to AI, visit rajeshkantubhukta.github.io. I look forward to connecting.

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